

CHAPTER THREE

3.0

PHYSICAL CARE

3.1 BED BATHING A PATIENT

It is a bath (or wash) given to a patient in the bed who is unable to care for himself.

Two categories of baths given to clients

- Cleansing bath
- Therapeutic bath

Before carrying out a bed bath the nurse should ensure that he or she has all the necessary equipment at hand. The nurse should never leave the patient exposed or at risk during the procedure to obtain missing equipment

Theoretical Basis

The theory that serves as the basis for the bed bath is Orem self-care theory. The proponent of this theory was Dorothea Orem. The theory is premised on (a) self-care, (b) self-care deficit and (c) nursing system model. The one that can be accepted for bed bath is the concept of self care deficit. It assumes that nursing is required when a patient is incapable or limited in the provision of continuous effective self care in an area such as inability to perform ADL, IADL etc.

Other theory that can still be applicable in bed bath is the Levine theory of conservation of energy.

Cleansing bath

Cleansing bath is given chiefly for cleansing or hygiene purposes and includes:

- * **Complete bed-bath:** The nurse bath the entire body of a dependent patient in bed.
- * **Self help bed –bath:** clients confined to bed are able to bath themselves with the help from the nurse for washing the back or any other areas where the client requires assistance.
- * **Parhal bath (Abbreviated Bath):** Only the parts of the body that might cause discomfort or odor of neglected are washed, these include face, hands, axial, perineum and back

*Tub bath : preferred to bed baths because it is easier to wash and rinse in a tub, can also be used for therapeutic baths

*shower: many ambulatory clients are capable of using shower.

Note:-

- (1) The water should feel comfortably warm for the client
- (2) People vary in their sensitivity to heat, generally, it should be between 43-46⁰c (110-115⁰f).
- (3) The water for a bed bath should be changed at least once
- (4) Before bathing a patient, determine
 - a. The type of bath the patient need
 - b. What assistance the patient need
 - c. Other care the client is receiving – to prevent an undue fatigue
 - d. The bed linen required.
- (5) When bathing a client with infection, the nurse should wear apron and gloves in the presence of body fluids or open lesion (Because of advent of HIV/AIDS and Ebola,, it must be a necessity.

3.2 Principles of Bathing a patient/client

- Close doors and windows: Air current increases loss of heat from the body of correction.
- Provide Privacy – Hygiene is a personal matter and the patient will be more comfortable
- The client will be more comfortable after voiding and voiding before bath is advisable.
- Place the bed in the high position: This will avoid undue strain on the nurses back.
- Assist the client to move near you: This facilitates access which avoids undue reaching and strain.
- Make a bath mitt with the wash cloth: It retains water and heat better than a cloth loosely held.
- Clean the eye from the inner canthus to the outer using separate corners of the wash cloth/face flannel – this prevents transmitting micro-organisms, prevents secretions from entering the nasolacrimal duct.
- Firm strokes from distal to proximal parts of the extremities increases venous return.

3.3 Purposes/Objectives of Bathing

- To remove transient moist, body secretions and excretions and dead skin cell

- To stimulate circulation
- To produce a sense of well-being
- To promote relaxation, comfort and cleanliness
- To prevent or eliminate unpleasant body odours
- To give an opportunity for the nurse to assess ill clients.
- To prevent pressure sores

3.4 Procedure for Bed bath

Equipment:

- Bed bath Trolley
- Hamper and bed strippers (if necessary)
- Screens

Top shelf of the Trolley

- A basin for mixing water
- Two jugs (one for cold and one for hot water)
- A tray containing powder, Vaseline, comb, brush, soap in the dish, shaving equipment (optimal).
- Lotion thermometer.

Bottom shelf of the Trolley

- Kidney receiver for hearing aids (where necessary)
- Bed protecting materials such as rubber sheet (optimal)
- Bath blanket (2)
- Bath towel (1)
- Wash clothes/face flannel (4 – 6)
- Clean pyjamas or gown
- Bed linens
- Incontinence pad (if needed)

2 Nurses carry out
Bed Bathing

3.5 Procedure guidelines for bed bathing a patient/client

STEPS	ACTION	RATIONALE
1	The patient's plan of care should be consulted before carrying out a bed-bath. This should include an assessment of the patient/client's needs and preferences and should address religious and cultural beliefs.	This is to plan the care and ensure the patient/client's comfort and safety during the procedure. It will equally promote the client's dignity and uphold his or her human right.
2.	Before each step of the bed-bath, the client's independence and level of participation should be assessed. Where possible, the patient should be encouraged and enable to carry out his or her own personal hygiene needs with the nursing assisting as required.	This will assist in determine the level of client's improvement and the promotion of independence.
3.	During each step of the procedure the nurse should ensure that correct manual handling techniques/and or equipment are use	This will reduce the chance or the risk of the patient/client or nurse sustaining a manual injury
4.	Hearing aids, dentures or spectacles should be removed and cleaned	This will ensure that prosthesis is in good working order and free from debris and contaminants.

5.	The procedure should be explained to the patient/client	This will ensure that the patient/client understands the procedure and gives his or her consent.
6.	The patient/client should be given the option of using urinal, bedpan or commode	This will reduce any disruption to the procedure and prevent any discomfort.
7.	All equipment required should be collected and arranged on the trolley and wheel to the patient bed side	It will minimize time spent away from the client during the procedure
8.	The environment around the patient/client should be prepared. Draw the bed curtains or bed screen, close the windows and doors.	It will confer on the patient/client privacy and dignity. The environment is made save.
9.	A wash bowl/basin should be filled with hot water and add cold water. Use lotion thermometer to assess the nature of the water, the nurse can use his/her elbow to feel it or ask patient/client to do it, if he/she can.	This will maintain the safety and comfort of the client
10.	The patient is assisted with the removal of clothing. The client is	This will maintain patient's modesty

	covered with one of the bed blanket and the second one is put on the bottom sheet.	and sustain body temperature.
11.	The patient/client should be asked whether he or she uses soap on the face. The face, neck and ears should be washed, with one of the face flannels, then rinsed and dried.	It promotes cleanliness and ensure the client's preferences are acknowledged.
12.	The top half of the body, including the hands, arms, axillae and torso, should be washed and rinsed. It is important to ensure that the area under the breasts is washed and dried thoroughly. Areas of the body that are not being washed should remain covered. Care should be taken not to wet drains, dressings and/or intravenous devices. Toiletries should be applied as required.	This will promote patient well being and cleanliness and reduce the risk of cross-contamination.
13.	The water should be changed and disposable gloves should be worn (if they are not worn before). Another face flannel is taken, the patient should be informed that the area around the genitalia is going to be washed, but if the client can do it by	This will reduce the risk of cross-contamination and to maintain client's comfort and dignity. This will promote good mental hygiene and reduce the risk

	herself/himself, he/she is allowed to do it. Verbal consent from the patient should be obtained. The flannel used for genitalia should be discarded and gloves removed and disposed off. When washing this area, it is important to remember that female should be washed from up/downward method. If the patient has a urinary catheter appropriate catheter hygiene should be carried out.	of cross-contamination.
14.	Change the water and face flannel	This will prevent cross-contamination.
15.	The legs and feet should be washed and dried. <u>All areas that are of being washed should remain covered.</u>	This will ensure cleanliness and maintenance of privacy.
16.	The patient should be assisted to roll onto his/her side. The patient's back should be washed, rinse and dried. Attention should be paid to sacral area, <u>observation to the pressure areas</u> should be done.	This will maintain the safety of the patient and ensure safe manual handling, and reduce the risk of cross-contamination.

17.	The basin of water is put under the legs of the patient and rinse with water and dry	This will ensure refreshing nature of the bath and allow circulation of blood.
18.	Clothes/Pyjamas should be replaced. If the patient is to retain bed, the bottom sheet and top sheet should be changed (if dirty or have not been changed that day) while the patient is being turned. <u>A minimum of two</u> nurses should be present during the procedure.	It will make the patient to be comfortable
19.	If the need arises, the patient should be assisted with facial shaving or the application of make-up	It will promote positive body image
20.	If the patients mouth/teeth has not been washed before bed-bath, the mouth/teeth is washed, dentures cleaned (where it is used)	Oral hygiene will be provided
21.	The patient's hair should be brushed or combed as desired	This will promote a positive body image and enhance patient's comfort
22.	The patient should be assisted to sit or lie in the desired position	This will reduce the risk of pressure ulcer/sores development and enhance patient's comfort



Nursing Procedure Manual

23.	Equipment should be removed from the bedside. The patient's possessions should be put back in their appropriate place. The locker, bedside table and call bell should be placed within reach.	This will make patient develop sense of belonging and evidence of being cared for.
24.	Hands should be washed and any clinical waste should be disposed off. The trolley is moved back to the utility room. Hamper and bed strippers are removed.	This will promote a good environment.
25.	Document the procedure carried out and document your observations and report to the ward leader.	Encourage the provision of recorded documentation of care to aid communication with other members of the health care